Set your default browser to Chrome or IE11 to get the best Clinical Library experience. Learn how here

CLINICAL REFERENCE | SCAL
OCTOBER 2020

# Medical Management for Termination of First-Trimester Pregnancy and Early Pregnancy Loss with Mifepristone and Misoprostol

## RECOMMENDATIONS

Medical management for termination of pregnancy and early pregnancy loss (EPL) in the first trimester is recommended as an alternative option to surgery, for patients at/under 11 weeks and 0 days gestation.[1,3] Treatment with mifepristone and misoprostol is more effective than misoprostol alone in managing medical termination and EPL.[1] The mifepristone and misoprostol regimen is successful in over 95% of medical termination and 84% of EPL.[1-6] This Clinical Reference was developed by the Regional KP Southern California Family Planning Group based on the most current practice standards and the recommendations of the American College of Obstetricians and Gynecologists. It was approved by the specialty clinicians and members of the KP Southern California Chiefs of Obstetrics and Gynecology.

## MEDICAL MANAGEMENT FOR TERMINATION OF FIRST-TRIMESTER PREGNANCY AND EARLY PREGNANCY LOSS WITH MIFEPRISTONE AND MISOPROSTOL

**The mifepristone and misoprostol regimen is recommended as an alternative to surgical termination of pregnancy for patients with a confirmed first trimester intrauterine pregnancy measuring up to 11 weeks 0 days gestation, including multiple gestation, who have undergone the following work-up and do not have any of the contraindications stated below.**

***Recommended Work-Up*** *(See Figure 1)*

- A full history and physical along with ultrasonography and lab work should be conducted to ensure the patient is a candidate for medical abortion.
- Confirmation of intrauterine pregnancy and dating is based on a vaginal ultrasound exam, with documentation of specific findings including:
  - Evaluation of embryo/fetal number in uterine cavity
  - Measurements to document gestational age is at or under 11 weeks and 0 days
  - Evaluation of pregnancy landmarks, yolk sac, or the presence/absence of fetal/embryonic cardiac activity
- Counseling and informed consent are recommended per Patient Counseling and Informed Consent sections below.
- Administration of the following medication regimen typically begins in the clinic/office:
  - **Mifepristone (Mifeprex) 200 mg (1 tablet)**
    - Oral administration
  - **Misoprostol 800 mcg vaginal or buccal (4 tablets)**
    - Patient-administered at 24-48 hours following mifepristone administration

        - **For gestational age up to 8 weeks and 6 days:** one dose of 800 mcg misoprostol vaginally or buccally 24-48 hours following the initial mifepristone dose is recommended.
        - **For gestational age 9 weeks 0 days up to 11 weeks 0 days:** a repeat dose of misoprostol 800 mcg buccally used 4 hours after the first dose of misoprostol is recommended.[6]
    - **Additional prescription medications to consider**
        - NSAID and/or acetaminophen and/or narcotic as directed for pain control
        - Antiemetic as directed
        - Anti-diarrheal as directed
        - No clear recommendations exist for prophylactic antibiotics in studies. Antibiotic use is at the discretion of the provider.
- Verbal and written instructions are recommended for expected symptoms at home and precautions to go to the emergency room. The patient is to be provided a 24-hour emergency contact number (typically the call center for the respective medical center). See KPSCAL AVS for Medical Abortion.
- An in-clinic follow-up is recommended in 1-2 weeks, to confirm completion of abortion by ultrasound.
    - In the setting of a patient who is unable to return or who strongly prefers lab follow up for medical termination of pregnancy, the patient should have a baseline human chorionic gonadotropin (hCG) drawn the day of her counseling visit. The patient should then be scheduled for a telephone visit in 1 week (See Telephone Follow-up for details). The patient can either:
        1. Have a serum hCG lab draw in 1 week to confirm down trending hCG level (≥80% decline); or,
        2. High sensitivity urine pregnancy test (UPT) at 4 weeks.
        3. If managing EPL, transvaginal ultrasound should be the first line for follow up. However, if the patient cannot return for a clinic ultrasound then the serial hCG or UPT follow up may be made available. See Interim COVID 19 Guidance for Family Planning.
- Contraception counseling is recommended for all patients undergoing medical termination of pregnancy.
    - Birth control pills, patches and vaginal rings may be prescribed and initiated the day following misoprostol administration.
    - IUDs may be placed at the follow-up visit, unless there is an ongoing pregnancy. IUDs can only be placed after complete abortion is confirmed.
    - Injectable medroxyprogesterone acetate (Depo Provera) and contraceptive implant may be provided at follow up appointment, even in the setting of incomplete abortions.
    - Permanent contraception (via hysteroscopy or laparoscopy) may be offered and the state consent signed the day completion of abortion has been confirmed. Plans for a procedure may be arranged at least 4 weeks from the date of abortion for laparoscopic tubal ligation and 8 weeks for hysteroscopic tubal occlusion.

***Contraindications for medical termination of pregnancy with mifepristone and misoprostol include but are not limited to the following:***

- History of chronic adrenal failure or chronic steroid use
- History of coagulopathy or currently undergoing anticoagulation treatment
- Pregnancy of unknown location or ectopic pregnancy
- Unreliable for follow up

- Hemoglobin of <9.5 g/dL
- Known allergy to either mifepristone or misoprostol
- Patient is unwilling or unable to have IUD removal before initiating mifepristone treatment
- Inherited porphyria

***Patient Counseling:***

- The following counseling topics are recommended to be delivered to the patient by the treating physician:
  - Efficacy: over 95% successful outcome for medication termination, and 84% successful outcome for treatment of EPL (See Table 1).[1,2]
  - Possible side effects including fever, nausea and vomiting, diarrhea, dizziness, headache (See Table 2).[1]
  - Risks including pain, bleeding, infection, rare allergic reaction, need for dilation and curettage for retained products of conception (<1% will need emergency curettage for excessive bleeding).[1]
  - Alternative treatment options including surgical procedures
  - Post abortion contraceptive options as needed
  - Social work or psychiatric therapy support as needed

***Informed consent:***

- DANCO Company mifepristone consent should be signed per DANCO Company protocol/FDA REMS.

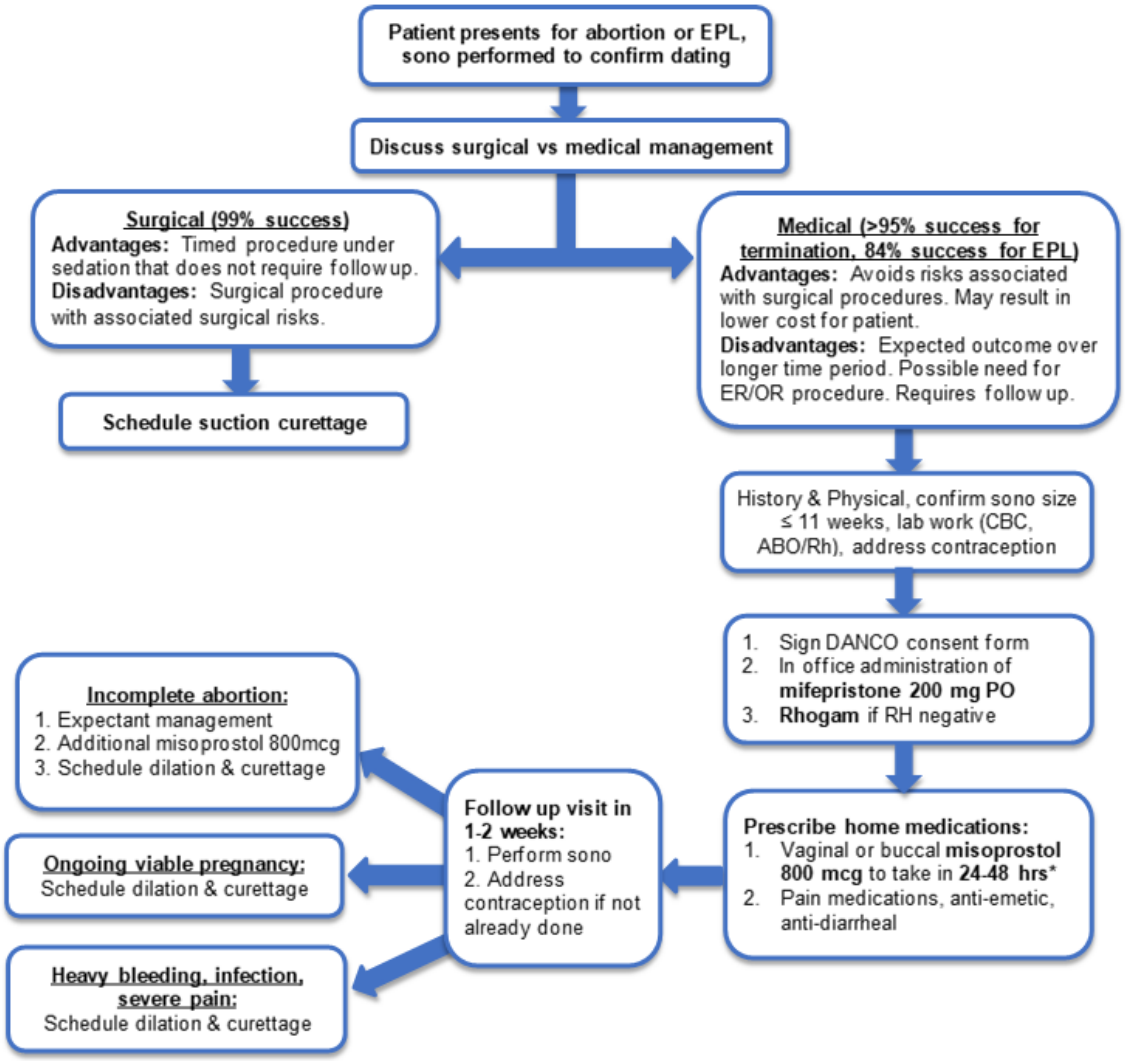
***Recommended Labs:***

- **Blood type:** ABO Rh documentation
  - **Rhogam** should be given to any patient with Rh negative results at time of mifepristone administration or within 72 hours of pregnancy termination.
- **Complete blood count** (CBC)

## MANAGEMENT OF INCOMPLETE ABORTIONS OR UNSUCCESSFUL OUTCOMES

- Incomplete abortion is defined as vaginal bleeding and/or pain with the cervix dilated, and/or products of conception found within the uterus or cervical canal on examination. The following treatment options are recommended for patients who are stable with incomplete abortions on follow-up ultrasound:
  - Expectant management
  - Dilation and curettage
  - Medical management with additional misoprostol, which may be offered up to 2 doses of misoprostol 800 mcg buccally or vaginally. This should be taken at least 12 hours apart. In patients who do not achieve completed abortion after 2 doses of misoprostol, recommendation is for a dilation and curettage.
- **Ongoing viable pregnancies** should be scheduled for a dilation and curettage.
- **Patients with heavy bleeding, severe pain or evidence of infection** should be scheduled for a dilation and curettage. This may be done in the clinic, emergency room, or operating room depending on patient presentation and staff availability.

## FIGURE 1. ALGORITHM FOR THE MEDICAL MANAGEMENT OF PREGNANCY TERMINATION IN THE FIRST TRIMESTER WITH MIFEPRISTONE-MISOPROSTOL




* **For gestational age up to 8 weeks and 6 days**: use one dose of 800 mcg misoprostol vaginally or buccally 24-48 hours following the initial mifepristone dose; **for gestational age 9 weeks 0 days up to 11 weeks 0 days**: a repeat dose of misoprostol 800 mcg buccally should be used 4 hours after the first dose.[6]

## TOOLKIT

- KPSCAL AVS for Medical Abortion
- Telephone Follow-up for Medical Abortion
- Interim COVID 19 Guidance for Family Planning
- Table 1. Reported Success Rates of Medical Management of Pregnancy
- Table 2. Adverse Effects in Selected North American Trials of Medical Abortion Regimens

## REFERENCES

1. ACOG Practice Bulletin "Medical Management of First Trimester Abortion," No. 143, March 2014.
2. Schreiber CA, Creinin MD, Atrio J, Sonalkar S, Ratcliffe SJ, Barnhart KT. Mifepristone Pretreatment for the Medical

Management of Early Pregnancy Loss. N Engl J Med. 2018 Jun 7;378(23):2161-2170.

3. Dzuba IG, Chong E, Curtiss H, Lichtenberg ES, Margarita E, Hernandez L, Nguyen NN, Patel A, Rzayeva G, Smith PS, Tsertvadze G, Winikoff B. A non-inferiority study of outpatient mifepristone-misoprostol medical abortion at 64–70 days and 71–77 days of gestation. Contraception. 101(2020) 302–308.
4. Gatter M, Cleland K, Nucatola D. Efficacy and safety of medical abortion using mifepristone and buccal misoprostol through 63 days. Contraception. 91 (2015) 269-273.
5. Abbas D, Chong E, Raymond E. Commentary: Outpatient medical abortion is safe and effective through 70 days gestation. Contraception 92 (2015) 197-199.
6. Dzuba IG, Castillo PW, Bousieguez M, Hernandez EML, Vivar JJC, Smith PS. A repeat dose of misoprostol 800 mcg following mifepristone for outpatient medical abortion at 64-70 and 71-77 days of gestation: A retrospective chart review. Contraception. Accepted 20 May 2020.

## ABOUT THIS RESOURCE:

**Specialty:** Obstetrics and Gynecology

**First Issued:** October 2020

**Last Reviewed/Revised:** N/A

**Methodology:** Clinical Reference

**Contact Information:**
Lynn Ngo, MD MPH
Obstetrics and Gynecology
San Diego Medical Center
SCAL Family Planning Group
619-641-2250

Asma Saraj, MD
Obstetrics and Gynecology
Baldwin Park Medical Center
SCAL Family Planning Group
626-851-6835

## DISCLAIMER

The Southern California Permanente Medical Group (SCPMG) Clinical References are documents offering information and clinical guidance to practitioners for specific medical conditions or the appropriate use of medical technologies in patient care. Clinical References are acceptable as reference or educational documents, but they generally do not meet the evidence standard or evaluation requirements defined for an evidence-based clinical practice guideline.

Clinical References are not intended to replace a clinician's judgment or to establish a protocol for all patients with a particular condition. It is understood that some patients will not fit the clinical conditions delineated within the reference documents, and that a Clinical Reference will rarely establish the only appropriate approach to a problem. Deviations from these references are appropriate in specific cases, yet exceptions to the references should be infrequent when there is strong direct medical evidence closely linked to health outcomes. In more controversial clinical subjects with weaker or indirect evidence, occasional exceptions are appropriate and anticipated.

Clinical References are not intended to be used as standards for utilization management or performance. SCPMG providers are responsible for applying recommendations to the specific clinical characteristics of each patient. In all clinical situations, SCPMG physicians have authority and autonomy in planning and directing the care of patients.